

JIMMA UNIVERSITY

COLLEGE OF HEALTH SCIENCE

DEPARTMENT OF NURSING AND MIDWIFERY

SOME IMPORTANT LICENSE EXAM FOR GRADUATING
MIDWIFERY STUDENTS (2016).

1. The normal range of fetal heart beat during the is

- A. 100-160 B. 120- 180 C. 100- 180 D. 120-160

2. While you are working at health center one laboring mother come with history of pushing down pain when you do vaginal examination , you feel cord that in front of fetal head ; the member is ruptured and the cord also seen . Based on this which types of cord prolapsed?

- A. Occult B. Overt C. Fore lying

3. While you are working at healthcenter one laboring mother come with history of pushing down pain when you do vaginal examination, you feel cord that in front of fetal head; the member is ruptured and the cord also seen. Based on this what is the management of this?

- A. Instrumental delivery B. Refer for C/S C. Wait for SVD D. All

4. While you are working at health center one laboring mother come with history of pushing down pain. After you have take history and done vaginal examination, you feel foot of the fetus, based on this what is the presentation?

- A. Complete breech B. Frank breech C. Footling

5. While you are working at health center one laboring mother come with history of pushing down pain. After you have take history and done vaginal examination, you feel foot of the fetus, based on this, what is the management?

A. wait SVD B. Instrumental delivery C. Refer for C/S D. None

6. While you are working at health center one laboring mother come with history of vaginal bleeding , when you ask her the bleeding started after she failed on the ground , what the cause of her problem?

A. Apruption placenta B. Placenta previa C. Vasa previa

7. The most common risk factor for Apruption placentais

A. Smoking B. Age less than 18 year C. Age greater than 35 years

D. Previous history

8. If one mother come to the Health center with the history vaginal bleeding at which you are working at MCH, when you have done physical examination, on abdominal palpation you feel severe abdominal tenderness, FHB is not reliable, the color bleeding is dark red, what is the cause of her?

A. Apruption placenta B. Placenta previa C. Vasa previa

9. The current modified ART drug for pregnant mother is

A. AZT +3TC +NVP B. TDF +3TC +EFV C. TDF- 3TC –EFV D. 3TC +AZT +TDF

10. The most common infection during peuriperium is

A. Mastatis B. Breast engorments C. Endometritis D. Chorio amiotis

11. The drug choice for the management of severe preeclampsia/eclampsia is

A. Diazepam B. Calcium gluconate C. Magnesium sulphate

12. The drug used to treat the toxicity of Magnesium sulphate

A. Diazepam B. Calcium gluconate C. Hyralize

13. If one pregnant mother comes to health center while you are working at the MCH. After you have done physical examination her BP 140/100 mmhg, on abdominal palpation she is 18 weeks pregnant then, what is the diagnosis of this mother?

A. Chronic hypertension B. Gestational hypertension C. eclampsia D. severe preeclampsia

14. W/ro Almaz comes to health center at which you are working as Midwives at MCH. She is 28 weeks of pregnancy. Her BP 150/100mmhg, PR 86/MIN on urine analysis there is no protein in the urine then what is her diagnosis?

A. Chronic hypertension B. Gestational hypertension C. eclampsia D. severe preeclampsia

15. W/ro Chaltu comes to health center at which you are working as Midwife at MCH Clinic on physical examination fundal height 30 weeks, BP 140/90 mmhg, PR 90 bpm, on urine analysis protein 2+, she has no convulsion, then what is her diagnosis?

A. Chronic hypertension B. Gestational hypertension C. Pregnancy induced hypertension D. Mild preeclampsia

16. On partograph full dilated cervix is how much cm?

A. 8 cm B. 10 cm C. 9 cm

17. To use partograph at what cm to start? A. 10 cm B. 4cm C. 3cm D. 5 cm

18.If the cervical dilation on the partograph is to left of alert line, then, what is the condition of this labour?

A. Unsatisfactorylabour B. Satisfactory labour C. Good prognosis of labour
D. none

19. Zero station is at

A. Pelvic inlet B. Schine spine C. Pelvic brim D. Out let

20. The sequence to delivery of fetus is 1. Move up ward, 2. Clean the mouth, 3. Delivery thehead, 4. Check the cord tie on the neck, 5. Move down ward, based on this

A. 2, 3, 5, 4,1 B. 3, 4,5,2,1 C.3,2,4,5, 1 D. 1, 2,3,4,5

21. During second stage of labour if you face muconium by vaginal examination in cephalic presentation, what do you do immediately after the delivery of the head?

A. Clean the mouth B. Check the cord on the neck C. Sucking the muconium

22. Why it is needed to give vitamin K for new born?

A. For the development of bacteria in small intestine B. For prevention of hemorrhage .

23. What is the maneuver of abdominal palpation during pregnancy?

- A. Loveset maneuver B. Leopold maneuver C. Burner marshal maneuver
D. Pinaldmanuevre

24. After delivery of fetal head, inscapes of the anterior shoulder behind syphispubis is

- A. Shoulder presentation B. Shoulder dystocia C. Hand prolapsed D.
None

25. What is the management of the above (Q 24) ?

- A. Loveseatmaneuver B MS maneuvers C. Supra pubic pressure D. C/S

26. Rh incompatibility is occurred

- A. If no synthesis of antibodies B. Synthesis of antibodies C. If no reaction is
formed

27 . Rh incompatibility is formed between?

- A. Rh positive mother and Rh negative father B. RH- mother and Rh- father
C. Rh negative mother and Rh negative fetus D. Rh negative mother and Rh
posive father

28. If one pregnant mother comes Health Centre for ANC follow up while you are working at MCH clinic as midwife on laboratory investigation for hgb , the result shows 10 g/dl, then, what is her diagnosis?

- A. Non anemic B. Anemia C. Syphylis

29. If one pregnant mother comes Health Centre for ANC follow up while you are working at MCH clinic as midwife on laboratory investigation for hgb , the result shows 10 g/dl, then, what is your plan for her?

A. Ferussulphate 325 mg as prophylaxis for one month B. Ferussulphate 325 mg po daily for one month C. Ferussulphate 325 mg PO TID for one month D. council on nutrition and send to home

30. W/roAylech is 30 year old comes Health Center for ANC follow up. She is 32 weeks of pregnant. Her BP 120/70mmhg, PR 88 bpm , on laboratory investigation , urine analysis ; Protein positive , hgb 9 g/dl and VDRL Positive, based on this what is her diagnosis?

A. Anemia and preeclampsia B. Eclampsia and syphilis C. Syphilis and preeclampsia
D. Anemia and syphilis

31. Which of the following immunization is can be given at the birth?

A. BCG B. OPV 0 C. BCG and OPV 0

32. BCG is given through

A. Intra dermal B. Subcutaneous C. IM D. IV

33. The types of breech presentation in which both thighs flexed at the hip and both legs flexed at the knee is

A. Frank breech B. Complete breech C. Footling breech D. Compound presentation

35. W/ro Ayelech delivered at home comes to the Hospital with her mother with the history of vaginal bleeding while you are working at Oby/gyne ward. When you take history, the bleeding is started immediately after the delivery of the fetus, the placenta was complete, the uterus was well contracted, and then what is the cause of bleeding?

A. Tissue B. Tone C. Thrombin D. Truama

36. Which drug is saving during pregnancy?

A. Doxycycline B. Norfloxaciline C. Magnesium sulphate D. Streptomycin

37. Which drug is given during pregnancy and prevent neural tubal defect?

A. FeSO₄ B. Folic acid C. Magnesium sulphate D. Insulin

38. Active third stage management includes: 1. Oxytocin 2. CCT 3. Uterine massage

A. 1, 2 B. 1,3 C. 2,3 D. 1,2,3

39. If one laboring mother comes to Health Center with history of second stage of labour while you are working at maternity ward, which laboratory request is must at that time?

A. Hgb B. VDRL C. PICT D. BG and Rh

40. When comparing medial episiotomy with medolateral episiotomy which one is false?

A. Medial episiotomy is less blood loss than medolateral episiotomy

B. Medial episiotomy heal in less time than medolateral episiotomy

C. Medial episiotomy is less pain full than med lateral episiotomy

D. Medial episiotomy is less extension to anal sphincter than medolateral episiotomy

41. If one mother starts ANC follow up and take TT1. She didn't come on her appointment for TT2 vaccine, which one is true about this?

A. Give TT2 B. Restart from TT1 C. Give TT3 D. Give TT4

42. If one pregnant mother comes to the Health Center with history of severe headache, severe epigastric pain, severe vomiting, with history blurred vision. She is 32 weeks pregnancy. On laboratory investigation, protein is 3+, Her BP 150/110mmhg, PR 96bpm, no convulsion. What is her diagnosis?

A. Eclampsia B. Severe preeclampsia C. Mild preeclampsia D. Epilepsy

42. W/ro Almaz 34 weeks pregnancy comes to the Health Center with history of severe headache, severe right upper quadrant pain, severe vomiting, with history blurred vision.

she has history of convulsion. On laboratory investigation, protein is 3+, Her BP 160/110mmhg, PR 96bpm, RR 17 breath per minute,. What is her diagnosis?

A. Eclampsia B. Severe preeclampsia C. Mild preeclampsia D. Hypoglycemia

43. Which stage of labour is more dangerous for fetus?

A. First stage of labour B. Second stage of labour C. Third stage of labour

44. W/roChaltu is 28 years old. She wants to use natural method of family planning. She adjusts her menstrual cycle for the first six months. She saw her menstrual cycle on the 1st month on December 28, 2015, on the 2nd month January 27, 2016, 3rd on February 29, 2016, 4th on March 25/2016, 5th month on April 30/2016 and on the last month May 26/2016. Based on this when does she protect from sexual intercourse to avoid unplanned pregnancy?

- A. 6_20 B. 7—20 C. 7---19 D. 6---19

45. A woman has undergone elective abortion one week ago. Now she comes to the clinic with the chief complaint of hemorrhage. In PE cervix is closed, uterine is contracted with no tenderness. Her temperature is normal. What is the best treatment?

- A-Doxy 100 mg bid for two weeks B. Again dilate and evacuate uterus
C observation and check of Hb and Hct D hormone therapy

46. W/roChaltu is 36 years old. She has used IUCD method of birth control. Unfortunately she being pregnant and test for HCG positive what is your plan for her?

- A. Remove IUCD B. abortion C. Continues pregnancy

47. Which of the following criteria used for using LAM for birth control?

- A. Baby must be less than six months B. Baby must be on EBF C. Mother's menses not returned D. All

48. From which you should start for sucking the new born?

- A. mouth B. Nose C. Mouth and nose

49. During resuscitating the new born for asphyxia the mask includes

- A. mouth and nose B. mouth and chin C. mouth, nose and chin D. Mouth, nose, and eyes

50. Which one of the following is not among the three delays contributing to maternal mortality

- A. delay in decision to seek care
B. Delay in reaching the care
C. Delay in receiving care
D. Delay in constructing the road

51. Which of the following landmarks is used for classifying the degree of uterine prolapse?

- A. Cervix B. Vagina C. Ischia spine D. Vulva

52. Which one of the following interventions is important in reducing MTCT during labor and delivery?

- A. Shorten second stage by using vacuum
B. Perform artificial rupture of membrane to hasten the first stage of labor
C. Follow labour using partograph and minimize vaginal examination.
D. Assist delivery by performing episiotomy.

53. W/ro Talile is 30 years old. She is 10 weeks pregnant when she presents at Shenen Gibe Hospitals while you are working at maternity ward with history of light vaginal bleeding. This is Talile's second pregnancy. The pregnancy was planned and wanted. After you have completed your assessment of Talile's and your main finding includes the following:

Talile's BP 100/70mmHg, her PR 92bpm her temperature is 37.4

She has no skin pallor. She has slight lower abdominal pain and light vaginal bleeding; her uterus is corresponding to the date; She has no uterine tenderness, no cervical motion tenderness. The cervix is closed

Based on the above what is Talile's diagnosis?

A. Threatened abortion B. Inevitable abortion C. Incomplete abortion

D. Missed abortion

54. While diagnosis of fistula by placing three cotton wool swabs in the vagina one above the other, and run methylene blue into the bladder, if the upper swabs wet, it shows _____

A. Uretero-vaginal fistula

B. Vesico-vaginal fistula

C. Urethro-vaginal fistula

D. Rectovaginal fistula

55. If the cervical carcinoma is strictly confined to cervix, then what is the stage of this cancer?

A. Stage I B. Stage II C. Stage III D. Stage IV

56. The most common site ectopic pregnancy can be ruptured:

A. Corneal

B. Ampulla

C. Isthmus

D. Ovary

57. During the diagnosis of ectopic pregnancy by Culdocentesis there is dark blood. This shows

A. Rupture ectopic pregnancy

B. Unrupture ectopic pregnancy

58. W/ro Tesfanesh is 20 years old. She has used calander method of birth control. She wants to be pregnant and goes to the Health center with her husband for knowing their status for HIV/AIDS. To which the prevention classifies the four pronged National Strategies of prevention of MTCT?

- A. Primary prevention of HIV infection
- B. Prevention of unintended pregnancies among HIV infected women
- C. Prevention of MTCT
- D. Treatment

59. The third stage of labour:

- A. The routine use of oxytocics has been shown to improve hemoglobin level
- B. Ergometrine should be routinely used in all patients in the third stage
- C. Ergometrine may be used safely in management of the third stage of labour
- D. The incidence if retained placenta after vaginal delivery is 2 percent Waiting 60 min rather than 30 min will have the number of women requiring anesthetics for manual removal.

60. W/ro Badhatu is 35 years old. She is gravid a VI, para IV, she is 24 weeks of pregnancy based on LMNP come to health center with history of absence fetal movement. The uterus is not corresponding to date, there is no vaginal bleeding, and cervix is closed, based on this what is her diagnosis?

- A. Inevitable abortion
- B. Complete abortion
- C. Missed abortion
- D. Threatened abortion

61. W/ro Badhatu is 35 years old. She is gravid a VI, para IV, she is 24 weeks of pregnancy based on LMNP come to health center with history of absence fetal movement. The uterus is not corresponding to date, there is no vaginal bleeding, and cervix is closed, based on this what is the management of her problem?

- A. Give medical abortion method
- B. Induction
- C. Manual vacuum aspiration
- D. Follow the pregnancy

62. Which of the following are medical methods of abortion management?

A. misoprostone and mifepristone B. Mifepristone C. oxytocine

63.